

St James's Hospital — Emergency Department

Date: 09 Nov 2024

DISCHARGE SUMMARY

Patient: Lauren Bennett

DOB: 1991-04-18 (age 33)

NHS Number: 999 900 0009

Address: 4 Pelham Court, Leeds, LS6 2BN

Presentation: Self-presented with 2-day history of worsening shortness of breath and audible wheeze. Increased salbutamol use (every 2 hours) with diminishing response. Symptoms began following an upper respiratory tract infection 5 days ago.

Past Medical History:

- **Mild asthma** (childhood-onset)
- Allergic rhinitis

Examination on arrival: RR 26, SpO2 92% on air, HR 112, BP 124/76, afebrile. Chest: widespread polyphonic wheeze, prolonged expiratory phase, accessory muscle use. PEFr 180 L/min on arrival (39% of personal best).

Diagnosis: Acute moderate asthma exacerbation, viral-triggered.

Treatment given: Salbutamol 5 mg nebulised x2 with ipratropium 500 mcg added to second neb. Oral prednisolone 40 mg administered. Reassessed at 60 minutes: PEFr improved to 320 L/min, SpO2 97% on air, RR 20.

Allergies: NKDA.

Discharge medications:

- Prednisolone 40 mg OD for 5 days (3 days remaining)
- Salbutamol 100 mcg MDI — 2 puffs QDS regularly for 48 hours then PRN
- **Started beclometasone 200 mcg BD today as new preventer inhaler**

Follow-up: GP review within 48 hours. Patient advised inhaler technique should be reviewed at GP appointment. No respiratory clinic follow-up arranged.

Dr P. Khan, Emergency Medicine Registrar